

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°071/MVB_24/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (48)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Lubunga, Makiso-Kisangani and Mangobo
Reporting date	July 24, 2026
Publication date	July 25, 2026

INDICATORS KEYS — CURRENT TO JULY 24, 2026

CONFIRMED CASES — 5 PROVINCES 3,075 +102 new cases (Ituri 87 · N-Kivu 9 – Haut-Uele 6)	CONFIRMED DEATHS · LEATHITIS 1,354 · (44.0%) +45 deaths (Ituri 35 · North Kivu 6 – Haut-Uele 4)	SUSPECTED CASES OF THE DAY 274 Ituri 198 · N-Kivu 64 · Tshopo 9 – Haut-Uele 3
PATIENTS IN ISOLATION 755 Ituri 576 · North Kivu 179	CURED — CUMULATIVE 556 +16 recovered (Ituri 12 – North Kivu 4)	NATIONAL CONTACT TRACKING 76.4% 11,100/ 14,525 views · Ituri 73.8% - Top-Uele 72.1% % - N-Kivu 83.1% - Tshopo 90.9%

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected. The total remains at 48 affected health zones. 140 in the five provinces, of which 47 remain in active transmission.
- **Coordination:** Official inauguration of the Kasenyi diagnostic laboratory in the Tchomia health zone in Ituri.
- **Case trends:** In the last 24 hours, 102 new confirmed cases, 45 confirmed deaths and 16 recoveries were recorded:
 - o **Ituri:** 87 new confirmed cases were notified, mainly in Bunia (26), Rwampara (25), Nizi (14), Lita (6), Mangala (6), Mongbwalu (6), Bambu (3) and Mahagi (1), 35 deaths among which 82.9% were community deaths.
 - o **In North Kivu,** 9 new confirmed cases were reported (3 in Katwa, 3 in Butembo, 2 in Musienene, and 1 in Kyondo), along with 6 deaths, 83.3% of which were community transmissions. The provincial total has reached 299 confirmed cases and 188 deaths, representing a case fatality rate of 62.9%.

o **Haut-Uélé** : 6 new confirmed cases (3 in Isiro and 3 in Wamba) and 2 deaths at the Ebola Treatment Center (ETC) have been reported.

The provincial total now stands at 35 confirmed cases and 19 deaths, representing a case fatality rate of 54.3%, spread across 5 of the province's 13 health zones.

o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The cumulative totals remain at 5 cases, 4 deaths and 1 recovery in Tshopo and 3 cases, including 1 death and 2 recoveries in South Kivu, where the Miti-Murhesa health zone has gone 59 days and the Tshopo province 10 days without a new confirmed case.

• **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 88.9% of cumulative cases and 85.3% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 48 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

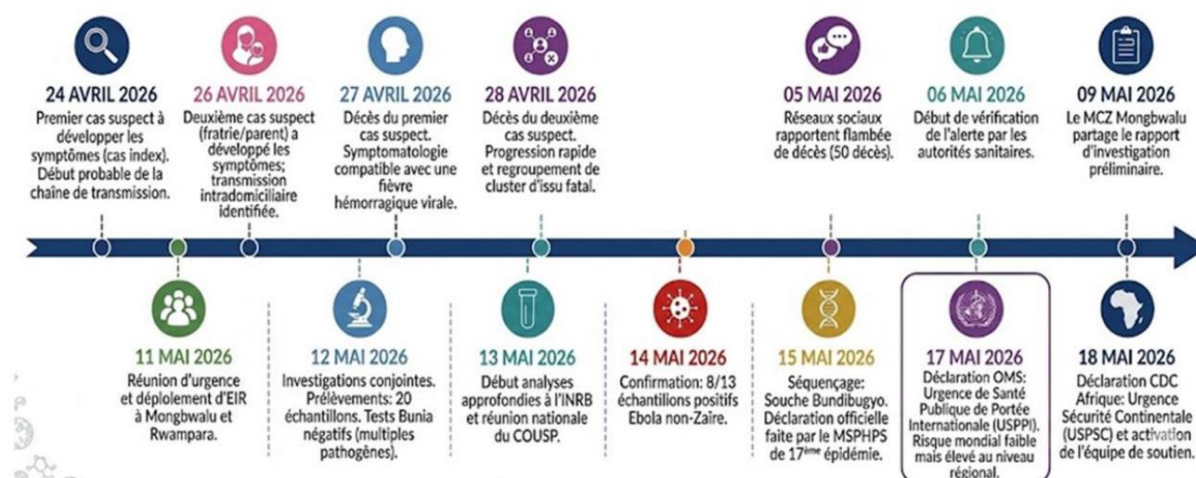


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,733	1,142	41.8%	28 / 36 (77.8%)	87
North Kivu	299	188	62.9%	11/34 (32.4%)	9
Haut-Uélé	35	19	54.3%	5/13 (38.5%)	6
Tshopo	5	4	80.0%	3/23 (13.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,075	1,354	44.0%	48 out of 140	102

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 88.9% of confirmed cases (n = 2,733) and 84.3% of deaths (n = 1,142), resulting in a case fatality rate of 41.8%. The health zones of Bunia (739 cases), Rwampara (515), Mongbwalu (490 cases), Nizi (330), Nyankunde (114), and Lita (109 cases) remain the most affected, accounting for approximately 84.0% of reported cases at the provincial level and 74.7% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,733	1,142	41.8%	87	29	6	35
Adja	10	1	10.0%				
Aru	5	1	20.0%				
Ariwara	7	1	14.3%				
Aungba	4	4	100.0%				
Bamboo	54	11	20.4%	3	1	0	1
Boga	1	1	100.0%				
Bunia	739	200	27.1%	26	12	3	15
Damascus	14	8	57.1%				
Drodro	6	4	66.7%				
Fataki	17	6	35.3%				
Gethy	1	0	0.0%				
Kambala	2	0	0.0%				
Kilo	23	11	47.8%				
Komanda	33	22	66.7%				
Lita	109	62	56.9%	6	3	0	3
Logo	9	3	33.3%				
Lolwa	4	1	25.0%				
Mahagi	2	1	50.0%	1	0	0	0
Mambasa	7	4	57.1%				
Mandima	13	9	69.2%				
Mangala	98	63	64.3%	6	3	0	3
Mongbwalu	490	232	47.3%	6	4	0	4
Nia Nia	78	45	57.7%	0	0	2	2
Nizi	330	158	47.9%	14	6	1	7
Nyankunde	114	30	26.3%				
Rimba	8	4	50.0%				
Rwampara	515	236	45.8%	25	0	0	0
Tchomia	40	24	60.0%				
North Kivu	299	188	62.9%	9	5	1	6
Blessed	42	29	69.0%				
Butembo	66	43	65.2%	3	2	0	2
Goma	1	0	0.0%				
Kalunguta	4	3	75.0%				
Katwa	136	88	64.7%	3	2	1	3
Kyondo	11	7	63.6%	1	0	0	0

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Mabalako	1	0	0.0%				
Masereka	4	0	0.0%				
Musienene	29	14	48.3%	2	1	0	1
Oicha	4	3	75.0%				
Vuhovi	1	1	100.0%				
South Kivu	3	1	33.3%	—			—
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	35	19	54.3%	6	2	2	4
Boma Mangbetu	6	3	50.0%	0	0	1	1
Isiro	11	7	75.0%	3	1	1	2
Pawa	7	5	71.4%				
Rungu	1	0	0.0%				
Wamba	10	4	42.9%	3	1	0	1
Tshopo	5	4	80.0%	—			—
Lubunga	1	0	0.0%				
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Total	3,075	1,354	44.0%	102	36	9	45

2.3 Mapping of confirmed cases

The spatial distribution confirms that transmission is mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Nyankunde and Lita, with transmission continuing in already affected areas, including the Rungu health zone in Haut-Uélé.

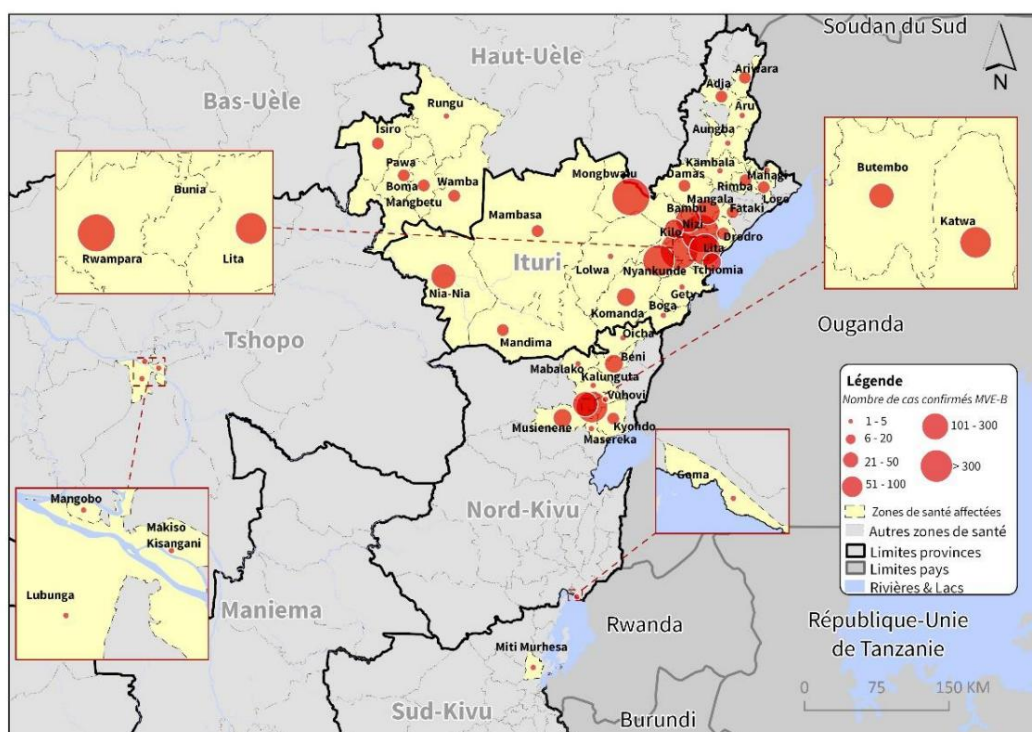
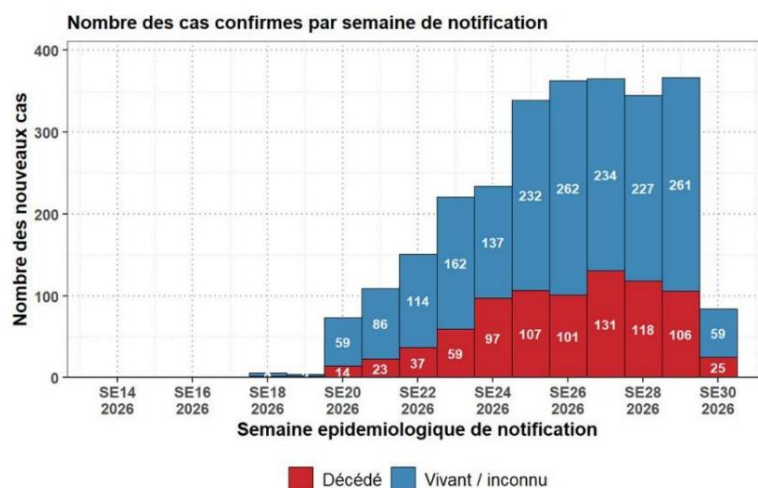


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

The continued transmission in the health sector in Rungu, Haut-Uélé, confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 661.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

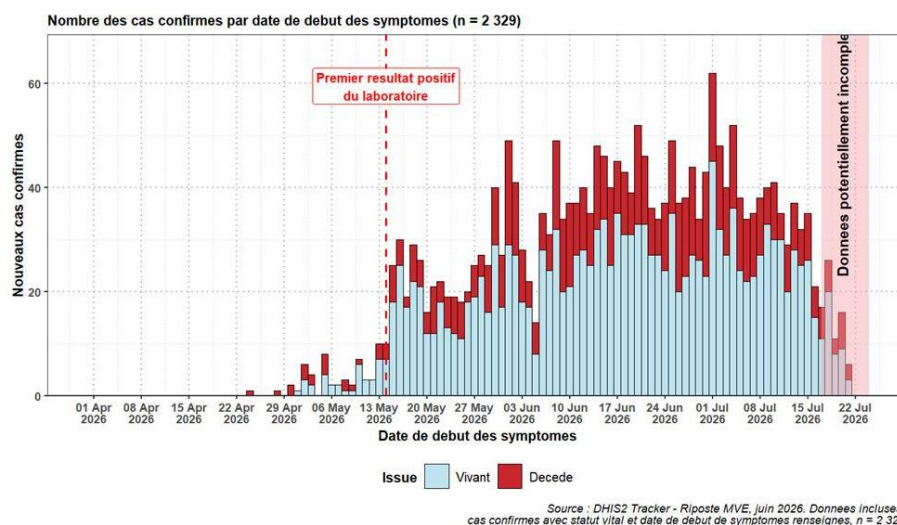


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

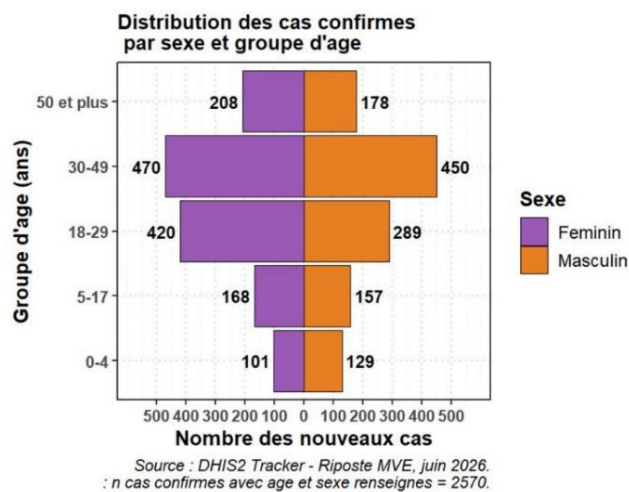
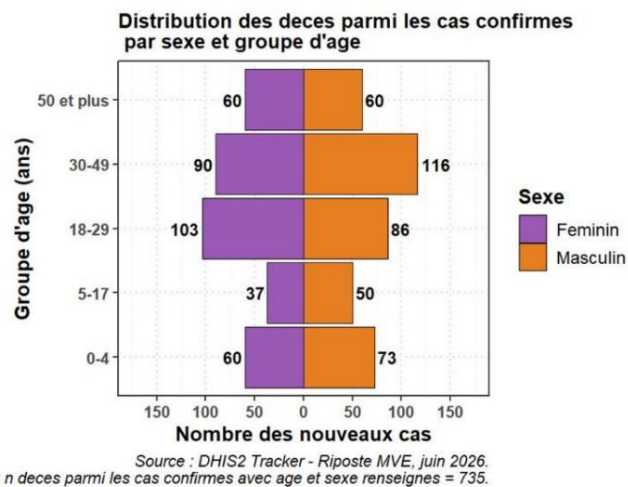


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	1	20	40	18	ND	79
New alerts received	865	957	5	31	ND	1,858
Total alerts for the day	866	977	45	49	ND	1937
Alerts investigated	641	948	4	24	ND	1,617
Investigation rate (24 h)	74.0%	97.0%	8.9%	48.9%	ND	83.5%
Validated alerts — alive	135	44	2	8	ND	189
Validated alerts — deceased (comm.)	63	20	1	1	ND	85
Total suspected cases for today	198	64	3	9	ND	274

In total, 1,937 alerts (including 1,858 new ones) were received, with 1,617 investigated within 24 hours (83.5%) and 274 confirmed (189 active cases and 85 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation delays and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

• **Ituri** : 240 samples documented in the collection network, of which 87 were positive (58 living and 29 deceased), representing a positivity rate apparent of 36.3%.

• **North Kivu** : 139 samples received and analyzed, of which 9 were positive (4 living and 5 dead), representing a positivity rate of 6.5%.

• **Tshopo** : 2 samples received and analyzed. All of these samples came back negative after analysis.

• *Official inauguration of the Kasenyi diagnostic laboratory in the Tchomia health zone in Ituri.*

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	10,123	7,466	73.8%
North Kivu*	4,111	3,418	83.1%
Tshopo	33	30	90.9%
Haut-Uélé	258	186	72.1%
South Kivu (end of monitoring)	0	0	N / A
Total	14,525	11,100	76.4%

* In Ituri and North Kivu, the monitoring rate was calculated only for health zones that had submitted reports. The health zones of Bambu, Gety, Damas, Lolwa, Mahagi, Rimba, Masereka, and Mutwanga did not have data available at the time of report consolidation.

Contact tracing remains insufficient at the national level (76.4%), below the operational threshold of 95%. Ituri only managed to follow up on 7,466 out of 10,123 contacts (73.8%). North Kivu achieved a performance of 3,418 out of 4,111 contacts. (83.1%). Haut-Uélé only followed up with 186 out of 258 contacts (72.1%). Tshopo maintains a relatively high performance with a 90.9% follow-up rate. Follow-up is closed in South Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 6 alerts notified

Six (6) alerts were notified to the PoE/PoC. All six active alerts were validated. Three (3) alerts were referred to the CTE, and 4 positive results were reported among the referred suspects.

North Kivu — 2 alerts notified

Two (2) lifeless bodies were intercepted at PoC Kasitu and referred for swab.

Tshopo — 1 alert notified

One (1) alert notified, validated at PoE Bangboka but invalidated at CTE Cinquantenaire.

Haut-Uele — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	Tshopo	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	4/20 (20%)	83.3% (80/96)
Number of people who went through PoE/PoC	10 4642	61,060	2,237	167,939
testing, % of travelers	94%	97.5%	100%	95.7%
screened, % of travelers who washed	94%	97.5%	99.5%	95.7%
their hands, % of travelers made aware of the issue	95.5%	97.5%	96.7%	96.5%
Number of alerts notified	6	2	1	9
Number of validated live alerts	6	0	1	7
Number of bodies recovered today	0	2	0	2
Number of problematic contacts intercepted today; %	0	0	0	0
of validated active alerts referred to CT/CTE	50%	0%	100.0%	57.1% (4/7)
% of lifeless bodies swabed	0.0%	100%	0.0%	100.0% (2/2)
Number of positive results of referred suspected cases	4	0	0	4

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator		Ituri	North Kivu Haut-Uele	Tshopo Together		
Number of beds		709	141	ND	ND	850
Patients in bed (Day -1)		570	186	ND	ND	756
Total admissions (24 hours)		79	34	ND	ND	113
Total admissions		649	220	ND	ND	869
Outings	Deceased	14	2	ND	0	16
	No case	40	34	ND	0	74
	Healed	12	4	ND	0	16
	Escapees	7	1	ND	0	8
	Transferred to the HGR	0	0	ND	0	0
Total outflows (24 h)		73	41	ND	ND	114
Patients in isolation (end of day),		576	179	ND	ND	755
including confirmed cases		275	16	ND	ND	291
including suspects		301	163	ND	ND	464
Occupancy rate		81.2%	126.9%	ND	ND	88.8%

In total, 755 patients were in isolation, including 291 confirmed cases and 464 suspected cases, for an overall occupancy rate of 88.8 % (755 patients for 850 beds), with 869 admissions and 114 discharges (16 recovered, 16 deceased, 74 non-cases, and 8 who escaped). Ituri accounts for the majority of the burden with 576 patients in isolation (275 confirmed, 301 suspected) out of 709 beds, representing an occupancy rate of 81.2%. In North Kivu, 179 patients are in isolation (16 confirmed, 163 suspected) for an occupancy rate of 126.9% (141 beds). Haut-Uele and Tshopo did not provide data at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	16 (94.1%)	1 (100%)
Confirmed cases are being monitored.	78 (56.1%)	10 (100%)
New suspected cases	16 (46.4%)	8 (100%)
Suspected cases are being monitored	93 (61.7%)	14 (100%)
Laboratory results announced (including positive ones)	33 (12)	12 (5)
Children separated — nursery / community	0/0	0/0
CTE support staff	198	10
Frontline Personnel (PPL)	10	6
Members of households and communities	31	0
EDS supported (SMSPS)	6	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVI

Health Zone	Confirmed cases	Cured. In hospital.		Death
Bunia	35	18	3	14
Rwampara	33	24	2	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	1	0	1	0
Total — case fatality	123	67	20	36

29.3% PPL = healthcare workers and frontline providers.

7 Pillar-Based Response Activities

COORDINATION The coordination meeting with the various partners was held.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Estimation of PCI input needs in the Framework for continuity of essential services and care; Analysis of indicators drawn from DHIS2 (ongoing); Development and Budgeting of the CSSE pillar plan.
Dignified and secure burials (DSM) ÿ Ituri: Closure of the training of the 2 EDS teams (Tchomia and Kasenyi); Holding of the planning meeting for the visits to the ZS of Mangala, Lita, Fataki, Tchomia and Adja; Start of the training of the 2 Niania and pk 51. ÿ North Kivu: Participation in the briefing session for managers of private fosas and in the two awareness sessions for women's associations and artists from the City of Butembo and Musienene on the MVE in order to solicit their commitment and ownership of the response actions.	PCI / WASH ÿ Ituri: 41 rings opened, 40 ESS monitored/accompanied, 13 IPC/ WASH assessments, 6 ESS and 17 means of transport decontaminated, 182 PPL trained and 43/58 households decontaminated within 48 hours. North Kivu: 2 ESS decontaminated, 8 ESS assessed, 65 ESS monitored/accompanied and 228 service providers briefed. Tshopo: CS Kwetu (16.1%) and CS Jamaa (37.1%) assessments, CSR St Joseph monitoring and HGR Tshopo triage. Haut-Uélé: decontamination of ESS and support for the rings around the cases. ÿ North Kivu : Decontamination of 2 health establishments for confirmed cases in the health zones of Butembo (HGR KITATUMBA and CH AFIA BORA); Evaluation of IPC scorecard in 8 health establishments including 1 in Beni, 4 in Butembo, and 3 in Katwa; Monitoring and support of 65 health establishments including: 30 in Beni, 6 in Butembo, 3 in Katwa, 2 in Kyondo and 24 in Oïcha; Briefing of 228 health establishment providers during formative supervisions including: 104 in Beni, 23 in Butembo, 9 in Katwa, 13 in Kyondo and 79 in Oïcha. Community deaths that benefited from a DHS (Death Health Survey): 9, including 5 in Beni, 4 in Butembo, and 1 in Oïcha. Community deaths that were the subject of SWAB (Swiss Blood and Blood Assistance): 17, including 5 in Beni, 8 in Butembo, 3 in Kondo, and 1 in Oïcha. Staff exposure risk assessment: 14, including 12 exposed to high risk in Butembo. ÿ Tshopo : Evaluation of CS Kwetu (16.1%) and CS Jamaa (37.1%); Monitoring and support of ESS (CSR St Joseph: 90% rate of implementation of recommendations made; Installation of triage service at HGR Tshopo.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 895 people were reached by the awareness messages carried out by the RECO 54 in households in 1/4 Health Zones (RWAMPARA); To date, the communities are beginning to apply hand washing without problems; 531 people were reached by the awareness themes during the educational talks held by community actors in health zone (RWAMPARA);	HOLISTIC CARE (HCC) ÿ Ituri: 12 new recoveries, 576 patients in isolation, 14 deaths among discharges; 1024 hot meals were served to patients including 121 to suspected cases, 122 to confirmed cases, 723 to PPLs and 58 to accompanying persons. ÿ North Kivu: continued monitoring of care in CTE/CT.

30 people, including village chiefs and leaders of women's associations, were engaged in response activities following community dialogue sessions held in the Health Zones; 633 people were reached during awareness-raising mass sessions held by community actors in the Health Zone. (MONGBWALU Health Zone, RWAMPARA). ŷ North Kivu: Briefing of private health facilities, community dialogues with military/police personnel, traditional healers, and community actors; 28,250 people reached through door-to-door outreach and 3,921 in public spaces. Tshopo: Reception/awareness-raising at Bangboka Airport, motorized caravan, engagement of motorcycle taxi drivers, training/briefing of community health workers and leaders. Ituri: Advocacy, discussions, mass awareness campaigns, collection of community alerts, and feedback management. ŷ Tshopo: 324 people were reached during door-to-door awareness campaigns at the weather station and SOTRACO bus station in the city of Kisangani; 15,868 people were reached during awareness campaigns (Mangobo, Kabondo, Tshopo, Lubunga, and Makioso health zones in Kisangani); Commitment of motorcycle taxi drivers to Ebola awareness campaigns.	ŷ Tshopo: 0 new admissions today. 4 suspected cases currently hospitalized at the CTE HC Kisangani. Second PCR test performed on 2 suspects (results pending). 1 operational ambulance based at Makiso General Referral Hospital.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Meeting of the national and provincial pillar with the PTF; deployment of CAP investigators in several ZS; continuation of the Partners and EBO-PEP clinical trials; anthropological publications in progress.	LOGISTICS ŷ Ituri: 38 vehicles and 12 ambulances (PNC, FARDC, HGR Bunia and 9 ZS) were made available to the different pillars to ensure the transfer of cases and the mobility of the pillars on the ground: CREC: 2; PCI: 6; Coordination: 4; Lab: 3; PEC: 4; PoE: 2; Operation: 1; Surveillance: 4; Research: 1; Security and safety: 2; Data management: 1; ZS BUNIA: 1; ZS RWAMPARA: 2, Nyankunde: 1; Mongbwalu: 3; Log: 1; Fuel supply for 6 COUSP vehicles, 332.5 liters and 2 SGI Provincial 150 liters and 40 liters DPS Generator, in total 522.5 liters. ŷ North Kivu: Joint logistics/administration-finance mission to Musienene, Butembo, and Katwa; delivery of infant formula and inventory of supplies. Tshopo: Delivery of IPC (Infection Prevention and Control) equipment and monitoring tools, fuel for ambulances/field vehicles/caravans, 4 vehicles, and 1 ambulance assigned to the peacekeeping teams. Ituri: Support for the mobility of the peacekeeping teams and inventory monitoring. ŷ Tshopo: 4 vehicles and 1 ambulance were made available to the various pillars to ensure the transfer of cases and the mobility of the pillars on the ground; Delivery of PCI devices to PK 23, to the office of the

DPS; Delivery of surveillance tools to the 4 ZS of the city (investigation, alert, contact tracing).

PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH)

Ituri: Continuation of activities on the ground.

8 Main challenges, impacts and actions required

Challenge No.		Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 126.9% in North Kivu (bed capacity: 141) and 81.2% in Ituri (576 patients for 709 beds);	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Contact tracking performance low in Ituri 73.8%; North-Kivu 83.1%; Haut-Uele 72.1% and 90.9% Tshopo	Uninterrupted transmission chains	Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risks (123 cases of PPL including 36 deaths)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Butembo and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of the pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Official inauguration of the Kasenyi diagnostic laboratory in the Tchomia health zone in Ituri



Closing of the briefing on the MVE (CREC and SBC) for community leaders and journalists from the Masereka health zone in North Kivu

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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